

CUI

PERFORMANCE WORK STATEMENT

FOR

AMBULANCE SERVICES

(NON-PERSONAL SERVICES)

AT

TYNDALL AIR FORCE BASE, FLORIDA

CUI

1.0 DESCRIPTION OF SERVICES

1.1 BACKGROUND

This contract is a Non-personal Health Care Services contract, as defined in FAR 37.101, under which the Contractor is an independent contractor. “Non-personal Services Contract” means a contract under which the personnel rendering the services are not subject, either by the contract’s terms or by the manner of its administration, to the supervision and control usually prevailing in relationships between the Government and its employees. The Government may evaluate the quality of professional and administrative services provided but retains no control over the medical or professional aspects of services rendered (e.g., professional judgments, diagnosis for specific medical treatment performance shall be according to the requirements contained in this Performance Work Statement [PWS] and the professional pre-hospital emergency medical service protocols applicable to state of Florida.) (Reference Appendix A).

1.2 SPECIFIC TASKS

1.2.1 General. Contractor shall furnish all management, supervision, equipment, vehicles, labor, training, and documentation to provide state certified Paramedic and Emergency Medical Responder (EMR) Services to respond to requests for emergency medical services, patient treatment, and stabilization as required. Contractor must provide at least one dedicated ambulance for this contract. The ambulances must be equipped to support Advanced Life Support (ALS). The Paramedic may be required to perform only basic first aid or any level beyond to include Advanced Life Support (ALS). The Contractor shall provide 24-hour, 7 days a week uninterrupted response coverage, to include response to in-flight emergencies (IFE’s). The contractor shall respond to medical incidents within Tyndall AFB (TAFB) property, all associated housing areas including Tyndall AFB family campground area and beaches. The contracted ambulance service response jurisdiction covers all areas US Highway 98 from the DuPont Bridge to the Mexico Beach city limits. The base is approximately 3,500 acres in size. The contracted ambulance service transfer boundaries are dictated by the availability of care within the civilian community. They shall respond to all calls per direction received primarily from 911 dispatcher, Tyndall AFB Fire Emergency Services or Security Forces. Attached in (Appendix G) shows the call tree of how 911 calls are processed. All calls shall have a minimum of two State of Florida certified/ National Registered Emergency Medical Technicians (one NREMR personnel, and one designated as a Paramedic). The receiving hospital shall be determined by patient needs, bed availability, transferring provider preference, and accepting physician. The patient transfer may need to be facilitated by the ambulance service attendant, and the transfer of care report should be communicated through the transporting ambulance attendants. No patients shall be transported back to the Tyndall Clinic as they must be taken to the receiving hospital off base. A Paramedic must remain with the patient until a qualified member of the receiving hospital properly accepts the patient. The Contractor shall provide all evaluations, treatments, and interventions as appropriate to the protocols of the State of Florida. The contractor must establish and maintain a system to ensure back up support is provided in the event of simultaneous responses, equipment failures, or other unforeseen circumstances. Services must always be available. All employees shall have required certifications by the State of Florida prior to performing services on this contract. Performance shall be according to the requirements contained in this Performance Work Statement (PWS) and professional standards of The Joint Commission on Accreditation of Healthcare Organizations, and Florida State Emergency Medical Service agencies.

1.2.2 TRAINING OF AMBULANCE PERSONNEL. All contract personnel performing under this contract shall have a high school diploma or general education equivalence certificate. The Contractor shall ensure that the Paramedics and EMR’s providing emergency response services to TAFB maintain current, unrestricted appropriate licensure and certification within the state. Additionally, paramedics must adhere to the recurring training and certification requirements in accordance with State

of Florida Medical Services Policies. The contractor must maintain files to assure that all EMRs performing under this contract have current certifications. Upon request the contractor must allow the Government access to review these documents for any EMR/Paramedic performing on the contract. Provide copies upon request.

1.2.3 Paramedics certified by the state of Florida shall continue to meet the minimum standards for Continuing Education Units (CEU) to remain current as prescribed in AFI-41-117, Medical Service Officer Education, dated 31 August 2023 CEU shall be obtained at no additional cost to the Government and shall be reported to the COR annually.

1.2.4 ORIENTATION. The Government supervisor shall ensure that all contract providers participate in the FW newcomer's orientation for newly assigned personnel to include regulations specific to their professional specialty hospital and Air Force policy and procedures within. Must be accomplished within 30 days of hire. The contractors must maintain their annual training requirements in accordance with Air Force and Medical Treatment Facility (MTF) regulations and guidance.

1.2.5 EMERGENCY AMBULANCE LICENSE. The Contractor shall ensure that all ambulance drivers performing services on Tyndall AFB are licensed to operate an emergency ambulance in the State of Florida; license to include an ambulance endorsement. Drivers must not have any reckless or dangerous driving citations within the previous 24-month period and have no Driving While Intoxicated (DWI) or Driving Under the Influence (DUI) convictions in the previous five years. Drivers can have a maximum of six points per year assessed under the state of Florida driving infraction system.

1.2.6 FIRST RESPONDERS. As a First Responder on TAFB the contractor is responsible for and shall obtain First Responder HAZMAT Training, at no charge to the Government. Additionally, during incidents involving Chemical, Biological, Radiological, Nuclear or High Yield Explosive (CBRNE) requiring decontamination, the contractor agrees to transport patients after gross decontamination at the scene has been accomplished. Contractor operational procedures regarding CBRNE casualty management and transportation shall be in accordance with State of Florida Health policy and local facility agreements. Contractor personnel must also complete airfield driving program training, provided by the Government. In an occasion in which the contractor is called to respond to an airfield emergency, they shall wait to be escorted onto the airfield by the appropriate military personnel if they have not received the airfield driving training.

1.2.7 PROCEDURE GUIDANCE. The Contractor shall perform procedures within the scope of care for which they are trained, based on the current United States Departments of the Army and Air Force Joint Prehospital Emergency Care Protocols. The Contractor shall follow the State of Florida Emergency Medical Services protocols. For aeromedical emergencies (IFEs), flight surgeon notification must be made in a timely manner by calling the Flight Surgeon On Call (FSOC). Flight surgeon consultation and guidance shall be provided (including flight line training) for every contractor responding to IFEs. In the event that transportation of a crew member of an in-flight emergency is required, the ambulance crew shall allow a flight surgeon to accompany the patient, if possible.

1.2.8 The Paramedic and EMR's shall perform additional duties such as minor clerical duties, committee members, and assistance in preparation for special events at no additional charge to the Government. These duties shall be performed during normal duty hours.

1.3 SPECIAL EVENTS & EXERCISES

1.3.1 SPECIAL EVENTS/SPECIALIZED TRAINING. The contractor shall provide additional

coverage as required to support special events. For historical estimates and information, see Appendix C. Hours for special events shall be ordered and funded as required. The contractor shall pre-position vehicle and paramedic at a mutually agreed location for all military sponsored special events and training.

1.3.2 EXERCISES. The contractor shall participate, as required, (approximately up to 12 per year) in exercises in support of the 325 FW and the 325 MDG. Exercise participation may include: Participating in advanced and post exercise meetings, respond with paramedics to exercise sites and perform mock duties such as mock field patient care and mock patient transport, follow the direction of the fire chief or on scene commander regarding the positioning of the ambulance, and provide detail exercise analysis as requested by the members of the 325 FW and MDG exercise evaluation team. Real World situations supersede exercise scenarios; therefore, no additional coverage is required during exercises. If the need for contractor participation is requested for exercise purposes, prior notification to exercise dates shall be given.

1.3.3 SERVICE COVERAGE FEES VS. TRANSPORT COVERAGE FEES. This service is a mechanism for Tyndall AFB to ensure it has full paramedic emergency response available for any medical emergency or in-flight emergency, and during special events and ceremonies. Payment shall be solely for the availability of such services and participation in 325 FW and MDG exercises. Actual patient transport fees shall be billed according to beneficiary category and location of transport. This PWS includes any transport from building 1465 (MDG). All claims, including all active-duty personnel shall be sent to the Managed Care Support Contractor (MCSC), Humana Military. Claims should be submitted electronically or mailed to TRICARE East Region Claims, Attn: New Claims, P.O Box 202146, Florence, SC 29502-2160. All transports shall be billed directly to the address above, or the individual's private insurance company for payment. Bills for these ambulance services shall be charged at the reasonable and customary rates, with payment accepted at the Tricare Maximum Allowable Charge (TMAC) Rate. The contractor must accept assignment of these claims and cannot balance bill the beneficiaries any amount above the TMAC amount. The Contractor's failure to obtain reimbursement, or collection of less than the billed amount, shall not financially obligate the 325th Medical Group, the United States Air Force, or the United States Government.

1.3.4 DOCUMENTATION. Prepare all documentation to meet or exceed established standards of the MDG to include timeliness, legibility, accuracy, content, and signature. The contractor ensures complete patient identifying information is on all documentation to be provided to the MDG relating to the transport. Run sheets and patient information shall be submitted electronically to the MDG by the 5th calendar day of the following month. (REQUIRED REPORT A001).

1.3.5 RECORDS. The Contractor personnel shall be responsible for creating, maintaining, and disposing of Government required records. If requested by the Government, the Contractor personnel shall provide the original record, or a reproducible copy, of any such records within one duty day or receipt of the ambulance response. The contractor is also responsible for furnishing all records related to their response times dictating the time the ambulance unit is notified by dispatch, the time arrived on scene, total response time, and code designation for the call.

PATIENT LISTS. Regardless of its development, all patient lists shall be treated as privileged information. Lists and/or names of patients shall not be disclosed to, or revealed in any way, for any use outside the 325 MDG without prior permission by the Chief of Medical Staff (SGH) for TAFB unless the patient is a part of the Armed Services. All transports of Service Members, including guard and reserve, must be reported to the Chief of Medical Staff, Command Post, and the Fire Department. Information must include the name of patient, status of patient, and location taken and will be reported immediately.

1.3.6 PATIENT SENSITIVITY. Contractor personnel shall respect and maintain the basic rights of patients, demonstrating concern for personal dignity and human relationships.

1.3.7 RELEASE OF MEDICAL INFORMATION. The contract personnel shall only release medical information obtained during the contract to destination civilian emergency room staff and to other 325 MDG staff involved in the care and treatment of that individual patient.

1.3.8 PATIENT PRIVACY/SECURITY. The contractor shall comply with all applicable aspects of the Health Insurance Portability and Accountability Act of 1996 to ensure the security and privacy of patient information. In addition to patient privacy, the contractor shall provide close hold on any information pertaining to base activities that are dispensed to the contractor in execution of their duties.

1.3.9 HEALTH INSURANCE PORTABILITY AND ACCOUNTABILITY ACT. Under this contract's PWS, the contractor shall be considered a 'Business Associate' under the terms of the Health Insurance Portability & Accountability Act of 1996 (HIPAA), Public Law 104-191, and its implementing regulations, including DoD 6025.18 "Privacy of Individually Identifiable health Information in DoD Health Care Programs". In the performance of this contract, the contractor shall when performing as a 'Business Associate' comply with all rules and requirements applicable to a 'Business Associate' under HIPAA and its implementing regulations, including DoD 6025.18. Violation of HIPAA may result in the imposition of fines and/or imprisonment. The contractor shall include the HIPAA notification and requirement in every subcontract for 'Business Associate' service.

1.4 GENERAL REQUIREMENTS

1.4.1 PERSONNEL

1.4.1.1 CONTRACT MANAGER. The contractor shall appoint a member of their management team who, irrespective of other responsibilities, shall have a defined responsibility and authority to make relevant project decisions. The name of this person and an alternate(s) who shall act for the contractor when the manager is absent shall be designated in writing to the Contracting Officer (CO) or his/her designated representative no later than the start of contract and within three (3) working days of any personnel change. (REQUIRED REPORT A002)

1.4.1.2 EMPLOYEES.

1.4.1.2.1 The contractor shall provide only properly trained, duty qualified paramedics and EMRs with experience in related work areas. They shall also be required to have a board-certified Emergency Medicine or Family Health Physician to serve as the ambulance crews Medical Director. Personnel staffing the ambulance shall have all applicable paramedic certifications required by the law in the State of Florida to include Advanced Cardiac Life Support (ACLS) and Pediatric Advanced Life Support (PALS). The Contractor shall ensure all certifications are current. The Contractor must take specific action including Primary Source Verification to ensure contract personnel have the required prerequisites for privileging and do not have disqualifying impediments

for privileges in the State of Florida prior to submitting credentials for hire.

1.4.1.2.2 In order to communicate with appropriate medical personnel, each Paramedic and EMR shall be able to read, write, and communicate clearly in the English language.

1.4.1.2.3 The contractor shall not employ any person who is an employee of the United States Government if the employment of that person would create a conflict of interest. Additionally, the contractor shall not employ any person who is an employee of the United States Air Force (USAF), either military or civilian, unless such person seeks and receives approval in accordance with the Joint Ethics Regulation, DoD 5500.7R. The contractor shall not employ any employee of the USAF if such employment would be contrary to Air Force policies and guidelines.

1.5 APPEARANCE. Contractor personnel shall present a neat professional appearance and be easily recognized as contractor employees. Employee clothing, accessories, and hair length shall be safe for the operations involved in contract performance.

1.6 STAFF OFFICE/SLEEPING QUARTERS. All offices, supply rooms, and sleeping quarter rooms shall be kept clean and organized within the infection control standards.

1.7 VEHICLE MAINTENANCE REQUIREMENTS. The contractor shall ensure preventive maintenance is performed on vehicles used to support this contract. Contractors must ensure the vehicle always remain locked while on base. All controlled medication must be secured, and temperature always controlled. The contractor shall ensure the vehicle and all medical equipment inside is operational. Contractors shall ensure all vehicles are clean and organized.

1.8 QUALITY ASSURANCE

1.8.1 QUALITY MANAGEMENT SYSTEM. The contractor shall develop and maintain an inspection system acceptable to the Government covering services under this contract. The contractor shall develop and implement procedures to identify and prevent defective services from recurring. As a minimum, the contractor shall develop quality control procedures that address the areas identified in the PWS and Service Summary. The contractor shall prepare a contractor quality control plan (CQCP) that shall describe the approach to developing and implementing a quality system that shall encompass all aspects of the contract. The contractor shall complete, implement, and make the CQCP available to the Government for review no later than 10 days after contract award. All subsequent changes to the CQCP must be coordinated with the 325 MDG COR and the CO within 10 days of changes. (REQUIRED REPORT A004)

1.8.2 The contractor shall render services in compliance with the standards, policies and procedures of the Florida Department of Health, Bureau of Emergency Management system providers. The Contractor shall, upon request, supply quality data and information related to services if support both the Performance

Improvement Program and Risk Management.

1.8.3 RESPONSE TIMES. The contractor must adhere to Department of Defense Instruction (DoDI) 6055.06 “DoD Fire and Emergency Services (F&ES) Program” Table E3.T1 Minimum Level of Service Objectives-Operations for ground response times. The contractor must be able to navigate the geographical landmarks to provide timely and effective pre-hospital emergency services. The contractor shall adhere to the following ground response times: For all codes requiring a transport unit with Basic Life Support (BLS) and Automated External Defibrillator (AED) capability, and/or ALS capability the response time shall not exceed: 12 minutes. This applies to all areas of TAFB property. Internal Clinic transports or calls should not exceed 5 minutes.

1.8.4 EXEMPTION POLICY. Per DoDI 6055.06 the minimum response times shall be on time for an average of 90% of all calls. The 90% threshold shall be calculated based on the aggregate of all responses annually, not including calls cancelled in route. As determined by the appointed COR, in some cases, late responses may be exempted from unsatisfactory performance reviews and response time compliance reports. These exemptions shall be for good cause only, as reasonably determined by the COR. The burden of proof that there is good cause for the exemption shall rest with the contractor. The contractor must file an extended emergency response report (located in appendix E) to request an exemption for a call in any month. This document shall be filed with the COR within five (5) days of the end of the previous month. Such request shall list the date, the time, and the specific circumstances causing the delayed response. The alleged good cause must have been a substantial factor in producing the excessive response time and must be documented in the exception report per EMS Policy. Good cause for an exemption may include, but is not limited to the following scenarios:

Inaccurate dispatch information when unedited records or tapes verify the following:

- a) Dispatcher gave incorrect call priority, address, or Thomas Brothers Map coordinates that had a negative effect on response time.
- b) Incorrect or inaccurate dispatch information received from a calling party or 911 Public Safety Answering Point.
- c) Disrupted voice or data transmission.
- d) Dispatcher failure to document/record times.
- e) Inability to locate address due to non-existent or inaccurate address.
- f) Unavoidable delay caused by traffic congestion when there is no reasonable alternate access to the incident.
- g) Weather conditions which impair visibility or create other unsafe driving conditions.
- h) Unavoidable delays caused by road construction and/or closure.
- i) Unavoidable delays caused by trains.
- j) Off-road or off-paved road locations.

- k) Unusual system overload.
- l) A declared state of emergency or disaster.
- m) Other situations requiring an exemption as judged by the COR

1.9 FREEDOM OF INFORMATION ACT (FOIA). All official Government records affected by this contract are subject to the provisions of the FOIA (5 U.S.C. 552/DoD 5400.7-R/AF Supplement). Any request received by the contractor for access/release of information from the records to the public (including Government/contractor employees acting as private citizens), whether oral or in writing shall be immediately brought to the attention of the COR. The COR, in turn, forward this information to Base FOIA Manager to ensure proper processing and compliance with the act.

1.10 COMPLAINT PROCESSES AND DENIAL/TERMINATION OF PRIVILEGES. Complaints validated by the COR and Functional Requirements Evaluator Designee (FRED) shall be reported in writing to the CO and the contractor for action. Failure of the contractor to correct validated complaints brought up by 325 MDG staff and the CO shall be considered a failure to perform and may result in an adverse performance appraisal that could lead to termination of contract.

1.11 EMERGENCY HEALTHCARE. TAFB has no Medical Emergency Services available. The Contractor shall be responsible for patient transportation for emergency medical care to the nearest appropriate and available Emergency Room.

1.12 INSURANCE REQUIREMENTS. Provide and maintain adequate liability insurance coverage consistent with the risks associated with the performance of all services required by this PWS. Minimum requirements are:

Commercial and General Liability and Malpractice	Excess Liability Umbrella
General Aggregate	Aggregate
Each Occurrence	\$2,000,000.00
Personal Injury	Each Occurrence
Medical Expense	\$1,000,000.00
	\$5,000.00

Refer to Federal Acquisition Regulation (FAR) Clause 52.237-7, Indemnification and Medical Liability Insurance.

1.13 PERFORMANCE OF SERVICES DURING CRISIS DECLARED BY THE NATIONAL COMMAND AUTHORITY OR OVERSEAS COMBATANT COMMANDER. Performance of Services during Crisis Declared by the National Command Authority or Overseas Combatant Commander. Patient Transport Services identified within this PWS are determined to be mission essential for performance during crisis according to DoDI 3020.27, Continuation of Essential DOD Contractor Services during Crises. Due to the critical nature of patient transport services, all services in this PWS shall continue during a declared crisis.

1.14 COMPLIANCE WITH FEDERAL AND FLORIDA OCCUPATIONAL SAFETY AND HEALTH ADMINISTRATION (OSHA)

1.14.1 The contractor shall comply with all applicable Federal and Florida OSHA requirements, where applicable. Additionally, the contractor shall safeguard USAF personnel and property during the conduct of their services, including compliance with base traffic regulations under normal circumstances.

1.14.2 It is the Contractor personnel's responsibility to report to the 325 MDG, Tricare Operations (TOPA) Office information necessary to assure hospital records can be maintained correctly and therefore complies with the Accreditation Association for Ambulatory Health Care (AAAHC), OSHA, and Center for Disease Control (CDC) health record requirements.

1.15 HEALTH REQUIREMENTS:

1.15.1 PREASSIGNMENT MEDICAL EVALUATION. No contract employee shall perform health care services under this contract unless a pre-assignment medical evaluation has been performed not more than thirty (30) calendar days prior to submission of credentials. The purpose of this medical evaluation is to determine if the individual, from a medical standpoint, possesses the minimum physical abilities needed to perform the proposed job without significant risk to personal health or the health and safety of others. As a minimum, the individual shall be free of physical defects or medical conditions which might reasonably be expected to place other workers, patients, or the public at risk.

1.15.2 MEDICAL EMPLOYEE IMMUNIZATION REQUIREMENTS. All incoming medical personnel must in-process through the 325 MDG Public Health the first duty day and prior to the commencement of performance of health care services under this contract. The Contractor shall provide to the Public Health (Medical Employee Health) Flight/COR certification that such employees have completed the medical evaluation requirements identified below. This certification shall state the date on which the examination was completed, the doctor's name that performed the examination and a statement concerning the physical health of the individual. If, in the opinion of the evaluating medical practitioner, the individual is found to be qualified for duty, the medical statement shall include the following wording: "(NAME OF EMPLOYEE) possesses the minimum physical abilities to perform the proposed duties of a (insert job title). He/she has documented proof of immunization against/immunity to rubeola, rubella, and hepatitis B and does not have an active infectious condition which might place others at risk." If, in the opinion of the evaluating medical practitioner, the individual does not possess the minimum capabilities needed to perform the proposed job or does not meet the conditions of employment or shall pose a risk to others, the medical statement shall contain the following wording: "For medical reasons, (NAME OF EMPLOYEE) is not qualified for employment as a (insert job title)." Certification shall be provided to the CO, who shall forward to the MDG Employee Health Office. The certification shall also contain the following statement: "(name of contract employee) is suffering from no contagious diseases to include but not limited to Tuberculosis, Hepatitis, and Venereal Disease." Per Center for Disease Control (CDC) guidelines, HCWs should be current or have serologic immunity for the following vaccine: HepB, HepA, Influenza, Measles, Mumps, Reubella (MMR), Varicella and Tetanus, Diphtheria and Pertussis (Tdap). Specific immunizations explained further:

1.15.3 MMR (immunization or titer levels for Rubella and Rubeola, must have either laboratory evidence of immunity (mumps IgG in serum, equivocal results shall be considered negative), or documented administration of 2 doses of live mumps virus-containing vaccine or laboratory confirmation of disease or born before 1957).

1.15.4 A history of chicken pox disease or positive titer (varicella vaccination must be accomplished if lack of immunity).

1.15.5 Baseline Tuberculosis (TB) skin test upon employment, unless documented as positive. New employees who have not been tested within the previous 12 months shall complete a 2-step baseline screening completed off base and result provided to public health.

1.15.6 A 3-dose Hepatitis B vaccination/series is required for all personnel who work directly with

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patients and who have occupational risk of exposure; declinations are not acceptable; this is a condition

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of employment. All contract employees who work in direct patient care shall require post-Hep B vaccination serologic testing to provide proof of immunity IAW CDC Guidelines for evaluating health care personnel for Hepatitis B virus protection and for administering postexposure management.

1.15.7 Influenza vaccine is required unless specifically exempted for religious purposes. Contract employees that have medical contraindications to any of the above-mentioned vaccines must provide medical documentation.

1.15.8 ANNUAL MEDICAL EVALUATION. All contract employees performing health care services under this contract shall complete annual medical evaluations as required by the MDG hospital employee health program. The contract shall provide to the COR a certificate, as described above, documenting the results of this evaluation.

1.15.9 PREVENTATIVE, PROPHYLACTIC, AND FOLLOW-UP PROCEDURES. The contractor shall ensure that his/her employees follow preventive, prophylactic and follow-up procedures, as well as infection control and employee health program procedures, as established by this PWS. This includes but is not limited to the annual flu vaccination. Required preventive, prophylactic and follow-up procedures shall be provided at the Contractors expense, the Contractor shall provide written verification of treatment.

1.15.10 Also, as a condition of employment, OSHA/AFOSHA requires that all contract personnel who shall have occupational exposure to blood or body fluids, or other potentially infectious materials, shall receive Hepatitis B vaccine, or provide documented proof of immunity to Hepatitis B infection.

1.16 ENVIRONMENTAL

1.16.1 ENVIRONMENTAL MANAGEMENT. The contractor shall comply with all applicable and the most stringent latest federal, state, county and local environmental laws and regulations to include, but not limited to: Air Force Instructions (AFIs), Defense Health Agency policies, guidelines, environmental compliance documents, and environmental management plans. The contractor shall maintain an awareness of changing environmental regulatory requirements to avoid any environmental deficiencies.

1.16.2 HAZARDOUS AND BIOLOGICAL WASTE REQUIREMENTS. All hazardous and biological waste generated or collected by the contractor shall be disposed of according to the 325 MDG policies and guidance.

1.16.3 SPILL AND RELEASE REQUIREMENTS. If applicable, the contractor shall comply with, but not limited to 325 FW Plans, Spill Prevention Control and Countermeasures Plan; Hazardous Waste Management Plan; Hazardous Material Management Plan; and Hazardous Materials (HAZMAT) Emergency Response Plan.

1.16.4 SPILL AND RELEASE NOTIFICATION. The contractor shall immediately notify the Tyndall AFB Fire department at (850) 283-4777 and the COR of a spill or release.

1.16.5 AFFIRMATIVE PROCUREMENT PROGRAM. The Contractor's affirmative procurement program shall use specified Comprehensive Procurement Guideline (CPG) materials with recycled and recovered content as the minimum standard. The Contractor shall also consider other green materials and products not listed, but commonly used by industry outside of the Government as a means of further reducing hazardous waste and solid waste. The Government's affirmative procurement programs are mandated to use recycled and recovered materials and products identified on the Environmental Protection Agency's CPG product lists (<http://www.epa.gov/cpg/products>).

1.17 SERVICE SUMMARY (SS). The SS defines the standard of performance for each listed service and

sets forth the maximum allowable deviation from standard performance for that service that may occur.

Performance Objectives	PWS Para	Performance Threshold	Method of Assessment
Specific Tasks	1.2.1-1.2.8	No more than one Deviation	Periodic review surveillance form
Maintains Qualifications	1.2.2-1.2.6	Mandatory prior to performance of duties. Zero tolerance for failure to maintain required qualifications	Periodic review surveillance form
Contractor shall provide 24-hour uninterrupted response coverage.	1.2.1	95% of coverage shall be provided to allow for transfers off base.	Periodic review surveillance form
The contractor shall adhere to the following ground response times: For all codes requiring a transport unit (BLS with AED) and or ALS capability the time shall not exceed: 12 minutes. Internal clinic transports, calls should not exceed 5 minutes. This standard applies to all areas of TAFB	1.8.4	Minimum response time shall be met or exceeded 90% of the time based on all responses annually (not including cancelled in route calls) unless properly justified as determined by the COR.	Periodic review surveillance form
The contractor shall provide monthly documentation to the 325 MDG relating to all transports.	1.3.4	95% of reports submitted by the 5th calendar day of following month.	Periodic review surveillance form
The contractor shall participate, as required, in Exercises and Special Events supporting the 325 FW and the 325 MDG.	1.3.1-1.3.2	95% coverage of each special event. No unjustified absences	Periodic review surveillance form
Patient/Customer Satisfaction	1.3.7, 1.10	No more than 1 valid complaint per month	Customer complaint surveillance form

1.18 GOVERNMENT- PROVIDED PROPERTY AND SERVICES

1.18.1 GENERAL. The Government shall provide the facilities, materials, and/or services listed here.

1.18.1.1 GOVERNMENT-FURNISHED FACILITIES. The Government shall provide a facility on TAFB for the contractor's use. The designated space will be suitable for supporting 24/7 operations, administrative tasks, and personnel rest. The specific room(s) will be identified and assigned by the Government prior to the contract start date. All space utilized by the contractor shall be kept organized and clean.

1.18.1.2 UTILITIES. The Government shall furnish electricity, water, sewage, and heating.

1.18.1.3 REFUSE COLLECTION. The Government shall provide dumpsters for refuse. The Government shall provide containers for collection of recycling materials. The contractor may not dispose of medical waste in these containers. Furthermore, the contractor shall be responsible for the proper disposal of all Medical Waste and Hazardous Waste.



1.18.1.4 PROTECTION SERVICES. The Government shall provide Security Forces and Fire Protection.

1.18.1.5 COMMUNICATION EQUIPMENT. The Government shall provide LMRs/Radios necessary to conduct operations in accordance with TAFB requirements.

1.18.1.6 The MDG shall provide a badge for access.

1.19 CONTRACTOR FURNISHED ITEMS AND SERVICES

1.19.1 GENERAL. Except for those items or services specifically stated in Section 1.18 to be Government furnished, the Contractor is responsible for furnishing everything required to perform this contract in accordance with all its terms. Items include Ambulances, paramedics, drivers, emergency medical technicians, non-transport vehicles, equipment, and road maps.

1.20 CONTRACTOR MANPOWER REPORTING. The contractor shall report ALL contractor labor hours (including subcontractor labor hours) required for performance of services provided under this contract for the United States Air Force via a secure data collection site. The contractor is required to completely fill in all required data fields using the following web address <http://www.ecmra.mil>.

1.20.1 Reporting inputs shall be for the labor executed during the period of performance during each Government fiscal year (FY), which runs October 1 through September 30. While inputs may be reported any time during the FY, all data shall be reported no later than October 31 of each calendar year. Contractors may direct questions to the ECMRA help desk.

1.20.2 Uses and Safeguarding of Information: Information from the secure website is proprietary in nature when the contract number and contractor identity are associated with the direct labor hours and direct labor dollars. At no time shall any data be released to the public with the contractor's name and contract number associated with the data.

APPENDIX A
PUBLICATIONS AND COMMERCIAL MANUALS

Publications and forms applicable to the PWS are listed below. The Contractor is obligated to follow these publications. These publications are available at <http://www.e-publishing.af.mil/> and maintained by AF publishing. The Government has coded publications as mandatory (M) or advisory (A). The contractor shall follow those publications coded as mandatory to the extent (that is, the specific procedure in a paragraph, section, chapter, or volume) specified in the PWS. The contractor shall be guided by those coded advisory to the extent necessary to meet requirements in this PWS. The Government may issue supplements or amendments to listed publications from any organizational level during the life of the contract. It is the responsibility to use the most current version of all publications. The contractor shall immediately implement those changes in publications that result in a decrease or no change in the contract price. Before implementing any such revision, supplement, or amendment that shall result in an increase in contract price, the contractor shall submit to the Contracting Officer (CO) a price proposal for approval. Price proposals shall be submitted within thirty (30) calendar days from the date the contractor receives notice of the revision, supplement, or amendment giving rise to the increase in cost of performance. The Government shall consider changes in the contract price due to supplements and amendments shall be considered under the FAR 52.212-4 "Contract Terms and Conditions-Commercial Items" clause. The Government shall continue to supply the Government forms needed for daily operations. Upon completion of the contract, the contractor shall return to the Government all issued publications and unused forms.

M = Mandatory Guidance, A = Advisory or Supplemental Information

The following documentation shall provide all needed background to offer insight and direction.

Number	Title	Date	Mandatory/Advisory
AETC Form 58	Civilian Identification Card		M
AF Form 1199	USAF Restricted Area Badge		M
AF Form 75	Visitor/Vehicle Pass		M
AFI 10-701	Operations Security (OPSEC)	23-Jul-19	M
AFI 33-332	Air Force Privacy Act Program	09-Mar-20	M
AFI 34-144	Child and Youth Programs	02-Oct-24	M
AFI 41-117	Medical Service Officer Education	30-Aug-23	M
AFI 44-119	Medical Quality Operations	17-Mar-15	M
AFI 44-176	Access To Care Continuum	21-Apr-20	M
AFI 71-101, Vol I	Criminal Investigations	30-Jun-19	M
AFI 71-101, Vol II	Protective Service Matters	20-May-19	M
AFI 44-102	Medical Care Management	22-Apr-20	M
AFMAN 17-1301	Computer Security	11-Feb-20	M

CUI

AFMAN 32-7002	Hazardous Materials Management	03-Feb-20	M
AFMAN41-209,	Medical Logistics Support	03-Jan-19	M
AFPD 16-14	Security Enterprise Governance	30-Dec-19	M
Florida Statutes, Chapter 401	Medical Telecommunications and Transportation	2023 version	M
DoDI 5200.48_DAFI 16-1403	Controlled Unclassified Information (CUI)	4-Oct-21	M
DAFI 31-101	Integrated Defense	01-Jun-23	M
DAFI 36-3026V1	Identification Cards for Members of the Uniformed Service, Their Family Members, and Other Eligible Personnel	31-May-23	M
DoD Directive 5400.7	Freedom of Information Act Program	05-Apr-19	M
DoDD Directive 5500.07	Standards of Conduct	15-May-24	M
DoDI 1402.05, Inc Change 1	Background Checks on Individuals in Child Care Services Programs	01-Jan-17	M
DoDI 5200.46, Inc Change 2, 2 Nov 20	DoD Investigative/Adjudicative Guidance for issuing CAC cards	02-Nov-20	M
DoDI 5200.48	Controlled Unclassified Information (CUI)	6-Mar-20	M
DoDM 1000.13, Inc Change 1, 28 Jan 20	DoD Identification (ID) cards Life-Cycle	23-Jan-14	M
DoDM 1402.05,	Background Checks on Individuals in DoD Child Development and Youth Programs	24-Jan-17	M
DoDM 5200.01 V1, DAFMAN 16-1404V1	Information Security Program: Overview Classification and Declassification	24-Feb-12	M
DODM 5200.02, DAFMAN 16-1405	Department of Air Force Security Program	28-Nov-23	M
MDGI 44-67	Infection Control Program	08-Dec-21	M
MDGI 48-1	Management of Dysbarism (decompression sickness)	19-May-22	M

CUI

CUI

MDGI 48-6	Aircraft Emergencies	11-Aug-22	M
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CUI

**APPENDIX B
ACRONYMS**

AAAHC	Accreditation Association for Ambulatory Health Care
ADC	After Date of Contract
AED	Automated External Defibrillator
AFB	Air Force Base
AFI	Air Force Instruction
ALS	Advanced Life Support
BLS	Basic Life Support
CDC	Center for Disease Control
CO	Contracting Officer
COR	Contracting Officer's Representative
DOD	Department of Defense
EMS	Emergency Medical Services
EMR	Emergency Medical Responder
FBI	Federal Bureau of Investigation
FOIA	Freedom of Information Act
HAZMAT	Hazardous Material
IAW	In Accordance With
IFE	In-Flight Emergencies
JCAHO	Joint Commission on Accreditation of HealthCare Organizations
OSHA	Occupational Safety and Health Administration
PWS	Performance Work Statement
SS	Service Summary
USAF	United States Air Force

APPENDIX C
WORKLOAD ESTIMATES

AVERAGE RUNS. The spreadsheet below represents the average number of runs per month over a 12-month period. Please note that these numbers are based on historical information and could vary.

MONTH/ YEAR	
Jan 25	19
Feb 25	12
Mar 25	27
Apr 25	21
May 25	21
Jun 25	29
Jul 25	25
Aug 25	15
Sept 25	19
Oct 25	22
Nov 25	11
Dec 25	15

SPECIAL EVENTS/SPECIALIZED TRAINING: The estimated number of event hours is an average of 8 per month. This number is based off historical information and is subject to change (Within the last twelve months, there have been a total of 96 event hours). Future event hours could fluctuate from this number dramatically.

**APPENDIX D
REQUIRED REPORTS**

<u>SEQUENCE NO.</u>	<u>TITLE</u>	<u>CONTRACT REFERENCE or PWS PARAGRAPH</u>	<u>AS OF DATE</u>	<u>FREQUENCY</u>	<u>REQUIRING OFFICE</u>
A001	Transport Documentation	1.3.4	5th calendar day of following month	Monthly	42 MDG/SGPJ/42 MDSS/SGGM
A002	Contract Manager Appointment	1.4.1.1	No later than start of contract and within 3 working days of personnel changes	Initial & As Required	42 MDSS/SGGM
A003	Contractor Quality Control Plan	1.8.1	10 days ADC	Initial & within 10 days of changes	40 MDSS/SGGM

ADC – After Date of Contract

APPENDIX E
TYNDALL AFB, 325 MEDICAL GROUP
EMERGENCY RESPONSE EXEMPTION REQUEST

This report must be completed and submitted with supporting documents for all exemption requests.

DATE:

CODE LEVEL:

TIME CALL RECEIVED:

DISPATCHED TIME:

ENROUTE TIME:

TIME ARRIVED SCENE:

LOCATION OF INCIDENT:

NATURE OF CALL:

WEATHER:

ROAD CONDITIONS:

DRIVER INFO:

INCIDENT NUMBER:

Reason for Extended Response Time: (Check all that apply)

1. ___ Dispatch error (Incorrect call priority, address, disrupted voice/data, or time record)
2. ___ Inability to locate address (Non-existent or inaccurate address)
3. ___ Unavoidable delay caused by traffic congestion without alternate route
4. ___ Extreme weather conditions with impaired visibility or unsafe driving condition
5. ___ Unavoidable delay by road construction and/or closure
6. ___ Unavoidable delay by train
7. ___ Unavoidable delay by off-road or off-paved locations
8. ___ Unusual system overload/extraordinary ALS fire demand
9. ___ Other

Detailed Explanation for Response Delay (Must be filled out/use additional sheets as needed)

Name/Title of Contractor Representative (Requestor)

REQUEST STATUS: APPROVED / DENIED (Circle one)

Reason: _____

Name/Title of 325 Medical Group Representative/Reviewer



APPENDIX F
TYNDALL AFB, 325 MEDICAL GROUP
SECURITY REQUIREMENTS FOR UNCLASSIFIED SERVICES – CAC REQUIRED

1. Contractor Notification Responsibilities: The contractor shall notify the contracting office within 30 days before on-base performance of the service. The notification shall include:

- a. Name, address, and telephone number of contractor representatives.
- b. The contract number and contracting agency.
- c. The location(s) of service performance and future performance, if known.
- d. The date service performance begins.
- e. Any change to information previously provided under this paragraph.

2. Obtaining and Retrieving Identification Media: As prescribed by the DAFFARS 5352.242-9000, *Contractor access to Air Force installations*, DAFFARS 5352.242-9001, *Common Access Cards (CAC) for Contractor Personnel*, FAR 52.204-9, *Personal Identity Verification of Contractor Personnel*, DoDI 5200.46, *DoD Investigative and Adjudicative Guidance for Issuing the Common Access Card* and DoDM 1000.13, *DoD Identification (ID) Cards, ID Card Life-Cycle*, the contractor must comply with the requirements set forth in these guidance.

a. The contractor shall obtain base identification and vehicle passes, if required, for all contractor personnel who make frequent visits to or perform work on the Air Force installation(s) cited in the contract. Contractor personnel are required to wear or prominently display installation identification badges or contractor-furnished, contractor identification badges while visiting or performing work on the installation.

b. The contractor shall submit a written request on company letterhead to the contracting officer listing the following: contract number, location of work site, start and stop dates, and names of employees and subcontractor employees needing access to the base. The letter shall also specify the individual(s) authorized to sign for a request for base identification credentials or vehicle passes. The contracting officer shall endorse the request and forward it to the issuing base pass and registration office or Security Forces for processing. When reporting to the registration office, the authorized contractor individual(s) should provide a valid driver's license, current vehicle registration, valid vehicle insurance certificate to obtain a vehicle pass. Government ID card holders may not use their credentials to sponsor an individual on to the base to perform work that's been contracted to an official contractor.

c. Common Access Cards (CACs). For contractors who require a CAC, contractors shall provide a listing of personnel who require a CAC to the contracting officer. The Government shall provide the contractor instruction on how to complete the Trusted Associate Sponsorship Systems (TASS) application and then notify the contractor when approved. Contractor personnel shall obtain a CAC from the nearest Real Time Automated Personnel Identification Documentation System (RAPIDS) Issuing Facility (typically the local Military Personnel Flight (MPF)). While visiting or performing work on installation(s)/location(s), contractor personnel shall wear or prominently display the CAC as required by the governing local policy. In order to be issued a CAC, the contractor must meet the requirements:



- A favorably adjudicated National Agency Check with Inquiries (NACI) (now a Tier 1 or T1 background investigation) or equivalent in accordance with revised Federal Investigative Standards is the minimum background investigation required for a final credentialing determination for a CAC.
- A CAC may be issued on an interim basis based on a favorable National Agency Check or a Federal Bureau of Investigation (FBI) National Criminal History Check (fingerprint check) adjudicated by appropriate approved automated procedures or by a trained Security or Human Resources (HR) specialist, and successful submission to the investigative service provider (ISP) of a NACI or a personnel security investigation (PSI) equal or greater in scope.

If the contractor does not meet the minimal criteria for CAC issuance, the COR shall work with the unit security assistant and 325 ABW/IP office to initiate the process on behalf of the contractor.

d. During performance of the service, the contractor shall be responsible for obtaining required identification for newly assigned personnel and for prompt return of credentials for any employee who no longer requires access to the work site.

e. When work under this contract requires unescorted entry to controlled or restricted areas, the contractor shall comply with DAFI 31-101, *Integrated Defense (ID)*, and [DoDM 5200.02 DAFMAN 16-1405](#), *Air Force Personnel Security Program*.

f. Upon completion or termination of the contract or expiration of the identification passes, the prime contractor shall ensure that all base identification passes issued to employees and subcontractor employees are returned to the issuing office.

g. All commercial vehicles will be directed to Gate 3 (Kelly St) or Gate 4 (Congressman Dickenson for Gunter) for processing through the Commercial Vehicle Inspection (CVI) Area, when operational. During non-duty hours and holidays, commercial vehicle inspections will be conducted at Gate 1 (Maxwell Blvd) or Gate 4 only. Inspection members will conduct thorough inspections of the interior and exterior of the vehicle for items prohibited from the installation such as explosive devices, weapons and ammunition, drugs, and open or closed alcohol containers.

h. Failure to comply with these requirements may result in withholding of final payment.

3. Child care Criminal History Background Checks: As prescribed by DoDM 1402.05, *Background Checks on Individuals in Department of Defense Child Development and Youth Programs*; Title 32 C.F.R., Part 86, *Background Checks on Individuals in DoD Child Care Service Programs*; Title 34 U.S.C., Subtitle II, Ch 203, Subchapter V, *Child Care Worker Employee Background Checks*; and AFI 34-144, *Child and Youth Programs*.

a. All contractor personnel reasonably expected to have regular contact with children on a DoW Installation, in a DoW Program, or as part of the military or DoW sanctioned activity, shall have a favorably adjudicated Tier 1 with child care background investigation. Contractor shall submit requests through the Contracting Officer Representative (COR) and will be processed by the 325 ABW/IP Office. Contractor shall be responsible to complete/certify the DD Form 2981 annually. Contractors must undergo a child care suitability reverification every five years.

b. Contractors must report immediately to their organization security assistant any subsequent automatic disqualification criteria IAW DoDM 1402.05, para 4.3, and presumptive disqualification criteria under para 4.4. Organization security assistant shall notify 42 ABW/IP Office within 72 hours of receiving notification.

4. Computer and Network Access Requirements: Contractor personnel that required access to unclassified Government computers and operations systems Information Technology (IT) Systems shall be designated as **Nonsensitive Positions**. Contractor personnel must have a Tier 1 and the Tier 1 be favorability adjudicated before operating **Government furnished** computer workstations or systems that have access to **Air Force** e-mail systems. These investigations shall be submitted by the Government at no additional cost to the contractor. *NOTE:* Since the background investigation process can take up to 18 months, contractor personnel may be issued a CAC before the process is completed after a favorable fingerprint return (3-5 working days) and investigation package submitted to the investigative service provider. Contractor personnel shall complete the current DoD IAA CyberAwareness Challenge course.

5. Freedom of Information Act Program (FOIA): The contractor shall comply with DODM 5400.07_AFMAN 33-302, *DoD Freedom of Information Act (FOIA) Program*, requirements. The regulation sets policy and procedures for the disclosure of records to the public. The contractor shall comply with AFI 33-332, *Air Force Privacy and Civil Liberties Program*, when collecting and maintaining information protected by the Privacy Act of 1974 authorized by Title 10, United States Code, section 8013. The contractor shall maintain records in AFI 33-322, *Records Management and Information Governance Program*; and disposed of in accordance with Air Force Records Information Management System (AFRIMS) Records Disposition Schedule (RDS) located at <https://www.my.af.mil/gcss-af61a/afrims/afrims/>.

6. Controlled Unclassified Information (CUI): The contractor shall comply with DoDI 5200.48_DAFI 16-1403, *Controlled Unclassified Information*, DoDM 5200.01v1_DAFMAN 16-1404v1, *Information Security Program*, and Freedom of Information Act Program. The contractor shall monitor CUI aggregation and compilation based on the potential to generate classified information pursuant to security classification guidance addressing the accumulation of unclassified data or information.

7. Reporting Requirements: The contractor shall comply with AFI 71-101, Volume-1, *Criminal Investigations*, and Volume-2, *Protective Service Matters*, requirements. Contractor personnel shall report to 325 Fighter Wing Information Protection Office, any information or circumstances of which they are aware may pose a threat to the security of DoD personnel, contractor personnel, resources, and classified or unclassified defense information. Contractor employees shall be briefed by their immediate supervisor upon initial on-base assignment and as required thereafter.

8. Physical Security: Areas controlled by contractor employees shall comply with base operations plans/instructions for FPCON procedures, Random Antiterrorism Measures (RAMS) and local search/identification requirements. The contractor shall safeguard all Government property, including controlled forms, provided for contractor use. At the close of each work period, Government training equipment, ground aerospace vehicles, facilities, support equipment, and other valuable materials shall be secured.

9. **Internal Operating Instructions:** The contractor shall adhere to the Air Force activity operating instructions (OI) for internal circulation control, protection of resources, and to regulate entry into Air Force controlled areas during normal, simulated, and actual emergency operations.

10. **Traffic Laws:** The contractor and their employees shall comply with base traffic regulations set forth in DAFI 31-218, *Motor Vehicle Traffic Supervision* and DoDM 5200.08v3_AFMAN 31-101v3, *Installation Perimeter Access Control*. Personnel in violation may be issued a Central Violations Bureau Form 1805 traffic ticket.

11. **Random Installation Entry/Exit Checks:** Entry/exit vehicle checks are conducted by order of the 325 FW Commander. These checks are conducted for the purpose of safeguarding the base and protecting Government property by discovering and seizing stolen property, classified information and contraband. Refusal to submit to an installation (entry/exit) vehicle check may result in the loss of base driving privileges, revocation of base registration, or debarment action.

12. **Cellular Phone Operation Policy:** The use of cellular phones while operating a motorized vehicle is prohibited on Tyndall AFB. Although discouraged, drivers are authorized to use devices, i.e. ear bud or ear boom, which allows their cellular phone to be operated hands-free. The device must not cover both ears. This policy applies to everyone driving on Tyndall AFB. Personnel in violation may be issued a Central Violations Bureau Form 1805 traffic ticket.

13. **Security Education and Training:** The contractor shall be required to participate in the Government's in-house and web-based security training program under the terms of the contract.

14. **Operation Security (OPSEC):** The contractor shall comply with installation and organizational OPSEC requirements in accordance with AFI 10-701 and applicable supplements. They shall also receive a local threat briefing.

15. **Wireless Electronic Devices:** The contractor shall not establish their own Information Technology (IT) systems or networks (Local Area Networks [LAN], Wide Area Network [WAN], Cellular phone/USB Modem as WAN, Wi-Fi as WAN, etc.), or camera system without the direct permission of the Program Manager and governing communications and responsible information systems office (325 CS).

16. **Firearms and Ammunition:** Transporting weapons or ammunition, concealed or otherwise, **IS NOT** permitted by any non-law enforcement personnel on Tyndall AFB at any time regardless of state issued concealed weapons permits. Violations may result in criminal prosecution under the applicable federal laws.

17. **Illegal Weapons.** The below weapons are considered illegal, unless specifically authorized by competent authority, and are prohibited on Tyndall AFB. Violations may result in criminal prosecution under the applicable federal laws.

- a. Switchblade knives or knives with any type of automatic blade release.
- b. An incendiary/explosive weapon (e.g., grenades, flash bangs).
- c. Fireworks
- d. Homemade mortars, aka "tennis ball launchers" or similar devices.